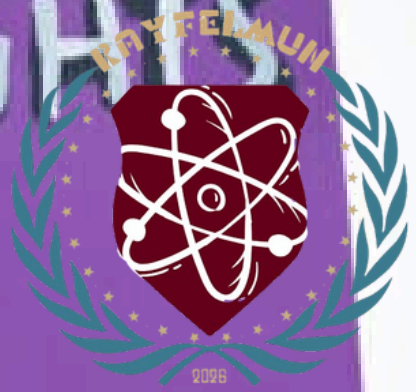


WOMEN'S RIGHTS
are
HUMAN RIGHTS



LEGAL

Rayfelmun

GA-6

Agenda Item: The Legal Status of Abortion and Reproductive Rights Under International Law

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1. Letter From the Head of Academy
Dear Delegates;

With great enthusiasm, I welcome you all to the second edition of our school's Model United Nations conference! As the Head of the Academy, I am honored to witness the extraordinary talent and dedication each of you brings to this platform for dialogue and collaboration. This year, we are excited to present six dynamic committees, each tasked with addressing some of the most significant global issues.

In the Legal Committee, delegates will engage with one of the most complex and sensitive topics of our time: the legalization of abortion. The issue of abortion has been the subject of intense debate in different parts of the world for many years, shaped around fundamental concepts such as individual bodily autonomy, the right to life, public health, and social responsibility. States have adopted various policies on this issue in line with cultural and legal differences, leading to different practices internationally.

Considering different perspectives within a framework of mutual respect, examining the effects of existing practices, and developing proposals that can provide solutions to the problems facing the international community are the most important elements of our committee. During our discussions, we will comprehensively address issues such as human rights, women's health, societal impacts, and legal regulations.

Thank you for being part of this journey. I wish you thoughtful discussions, meaningful connections, and an inspiring MUN experience. If you have any questions, my door is always open.

Sincere Regards,

Defne YAMAN

Head of Academy

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2. Letter From the Under Secretary General of LEGAL

Dear Delegates;

Welcome to RAYFELMUN '26! I am Ulaş GEGA, the Under Secretary General for the Legal Committee. It is a true privilege for me to be working alongside you in this committee. Together with the Legal Committee's academic team, we have conducted extensive research to draft a comprehensive study guide that is both easy to understand and highly engaging to debate.

In my view, the legalization of abortion and reproductive rights is one of the most striking and relevant topics of our time. Over the next three days, you, the delegates, will discuss the legal status of abortion, its profound impact on women, and the broader state of reproductive rights world wide. Because this is a topic capable of fostering a rich debate environment where everyone can bring at least some level of insight, I believe it is an excellent committee for first-timers. At the same time, it offers an incredible platform for our experienced delegates to truly stand out.

RAYFELMUN '25 held a very special place in my heart, but stepping up as a USG for the first time at RAYFELMUN '26 makes this year uniquely unforgettable for me. We sincerely hope that we all share a wonderful and productive three days together. Should you have any questions or require assistance, please do not hesitate to contact me at the email address provided below.

Sincerely,

Ulaş GEGA

Under Secretary General

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3. Introduction To the Committee:

The Legal Committee (LEGAL), the primary forum of the United Nations General Assembly tasked with the evaluation, progressive development, and codification of international law. This committee serves as the legal backbone of the United Nations, analyzing global issues through the strict lens of international jurisprudence, multilateral treaties, and the fundamental principle of state sovereignty.

4. Introduction to the Agenda Item:

a. Evaluation of Abortion Rights Under International Human Rights Law (IHRL)

The Right to Life vs. Reproductive Autonomy

The core of the international debate centers on balancing the legal protection of a fetus with a pregnant individual's bodily autonomy, health, and right to life. Under Article 6 of the International Covenant on Civil and Political Rights (ICCPR), the right to life is inherent, yet international human rights bodies consistently rule that this protection cannot be applied in a way that creates a direct threat to the life or health of the pregnant individual. State laws often clash when defining where the fetus's legal personhood begins versus where a woman's right to decide over her own body ends.

The Prohibition of Torture and Ill-Treatment

Denying access to safe, legal abortion services—especially when a pregnancy poses a severe threat to a person's physical or mental health—is increasingly recognized as a violation of absolute human rights standards. The UN Committee Against Torture (CAT) has noted that forcing individuals to carry unwanted pregnancies to term under traumatic circumstances, or driving them toward dangerous, unsafe "back-alley" procedures, causes immense physical and psychological suffering. Under international law, these restrictive state actions can amount to cruel, inhuman, or degrading treatment.

Discriminatory Frameworks

Because legal restrictions and criminal bans on abortion exclusively impact individuals who can become pregnant, they are heavily criticized as forms of gender-based discrimination under treaties like CEDAW and the ICCPR. These restrictive frameworks systematically limit women's equal access to comprehensive healthcare, which frequently derails their educational opportunities, financial independence, and workforce participation. When states fail to provide safe reproductive care, they perpetuate systemic gender inequality and violate their non-discrimination treaty obligations.

b. Healthy and Secure Ways of Abortion

According to World Health Organization (WHO) standards, abortion is a routine and essential healthcare procedure that is highly safe when executed using evidence-based medical practices tailored to the pregnancy's duration and assisted by individuals with the necessary skills. The evolution of reproductive science has established two primary secure methods: Medical Abortion, a highly effective, non-invasive protocol utilizing specific medical treatments that can be safely managed in early terms, and Surgical Abortion, which relies on safe techniques like vacuum aspiration to eliminate the severe risks of uterine perforation, hemorrhaging, and long-term infections associated with outdated methods. Despite these clinical advancements, systemic barriers such as a lack of trained healthcare providers, inadequate rural funding, supply chain disruptions, and the persistence of unregulated, hazardous "back-alley" facilities continue to place lives at immediate risk worldwide.

To combat this public health crisis and fully guarantee respect for human rights, international bodies advocate for member states to deploy multiple actions. Operationally, the United Nations Population Fund (UNFPA) serves as the leading UN agency on the ground, utilizing its strategic frameworks to end preventable maternal deaths by securing and distributing vital medical commodities to developing and crisis-affected nations. Concurrently, the Office of the United Nations High Commissioner for Human Rights (OHCHR) and the CEDAW Committee legally monitor state compliance, systematically framing the denial of these healthy, secure medical methods as a direct violation of international human rights law, bodily integrity, and non-discrimination treaties.

c. Abortion Tourism and Extraterritorial Jurisdiction

The dramatic divergence in global abortion laws has given rise to the phenomenon of "abortion tourism" (or reproductive travel), where individuals travel across state or national borders to access safe and legal abortion services that are restricted or banned in their home jurisdictions. While cross-border travel offers a vital alternative for healthcare, it has triggered intense geopolitical and legal conflicts over extraterritorial jurisdiction—the legal ability of a state to exercise criminal or civil authority over actions committed beyond its physical borders. This intersection of conflicting sovereign laws creates a highly volatile legal landscape for patients, healthcare networks, and international legal systems alike.

The Legal Conflict of Extraterritoriality

The core legal dispute arises when highly restrictive states attempt to extend their domestic criminal prohibitions beyond their borders, utilizing the nationality principle of international law to regulate the conduct of their citizens abroad. This legal overreach manifests in two distinct ways:

- ***Prosecution of the Citizen:*** Restrictive states explore or enact statutes designed to criminally charge their residents upon returning home if they obtained an abortion in a jurisdiction where the procedure is fully legal. This directly challenges the classic international law principle of ***double criminality***, which typically prevents a state from punishing a citizen for an act that was entirely lawful where it occurred.
- ***Targeting Aiding and Abetting:*** To paralyze cross-border networks, restrictive states create sweeping civil and criminal liabilities for anyone who facilitates reproductive travel. This includes prosecuting healthcare providers operating legally in the host nation, out-of-state funding organizations, or even family members who provide transportation. This long-arm jurisdiction directly clashes with the fundamental concept of territorial sovereignty, as one state attempts to dictate and penalize medical practices occurring inside the borders of another sovereign state.

Transnational Legal Vulnerabilities

The enforcement of extraterritorial bans creates severe operational vulnerabilities across digital, medical, and diplomatic spheres:

- ***Digital Surveillance and Data Privacy:*** Individuals seeking cross-border care leave extensive digital footprints. Restrictive states increasingly rely on data-mining, geo-fencing, and digital surveillance to prove a citizen traveled for an abortion. Authorities can subpoena search histories, period-tracking applications, location data from mobile phones, and financial records of out-of-state transactions. This turns everyday personal data into criminal evidence, bypassing traditional border checks through digital surveillance.
- ***The Chilling Effect on Healthcare Providers:*** The threat of cross-border civil lawsuits, asset forfeitures, or the revocation of medical licenses creates an immense chilling effect. Even if a doctor operates in a "safe" country, they may face arrest or legal harassment if they travel internationally or if their assets cross borders. This fear makes healthcare networks hesitant to openly accept, counsel, or treat out-of-state patients, narrowing the path to safe medical care.
- ***Interstate and International Friction:*** When one jurisdiction attempts to penalize activities legally performed inside another sovereign territory, it severely strains international legal cooperation. Restrictive states may weaponize Mutual Legal Assistance Treaties (MLATs) and extradition requests to force host states to hand over medical records or surrender accused individuals, forcing foreign courts to choose between diplomatic compliance and protecting human rights within their own borders.

d. Human Rights Mechanisms and Jurisprudence

International human rights standards are frequently interpreted and implemented through regional judicial bodies, creating distinct legal frameworks across different continents. Rather than relying on a singular global doctrine, the legal status of abortion is heavily shaped by regional treaties, court rulings, and the evolving jurisprudence of human rights systems in Europe, the Americas, and Africa.

The European Court of Human Rights (ECHR) does not recognize a standalone "right to abortion" under the European Convention on Human Rights. However, it mandates that Article 8 (Right to Respect for Private and Family Life) requires member states to provide clear, accessible, and effective procedures to access lawful abortion.

The legal frameworks governing reproductive rights vary significantly between the Inter-American and African human rights systems, representing opposite ends of the treaty spectrum regarding embryonic protection and bodily autonomy.

The Inter-American System: Governed primarily by the American Convention on Human Rights (San José Convention), this system features one of the most restrictive textual protections for unborn life in international law. Article 4(1) explicitly states that the right to life shall be protected by law, "and, in general, from the moment of conception." Despite this strict language, the Inter-American Court of Human Rights clarified in the landmark case **Artavia Murillo v. Costa Rica** that the protection of the fetus is gradual and incremental, meaning it cannot be used to completely strip women of their fundamental rights to health and personal integrity.

The African System: Conversely, the African continent hosts one of the most progressive and explicit treaty frameworks for reproductive autonomy via the Maputo Protocol (the Protocol to the African Charter on Human and Peoples' Rights on the Rights of Women in Africa). Under Article 14, the Maputo Protocol stands out as the first international human rights instrument to explicitly codify a woman's right to safe and legal abortion. It mandates that state parties must authorize abortion in specific exceptional circumstances, including cases of rape, incest, sexual assault, fetal abnormalities, and when the pregnancy imperils the mental or physical health or the life of the pregnant individual.

e. Potential Impacts of Legalization on Women's Lives

The legal status of abortion has a profound, multi-dimensional impact on women's lives, directly shaping their socioeconomic standing, long-term health outcomes, and fundamental bodily autonomy. Assessing these impacts is crucial for drafting effective policies, as the transition from a restrictive legal regime to a legalized framework creates sweeping changes that ripple across public health, labor markets, and educational institutions.

Socioeconomic Mobility and Educational Attainment

A critical dynamic of legalization is its direct correlation with women's long-term economic independence. When reproductive healthcare is legally accessible, women possess the agency to plan their families around their educational and career milestones.

- ***The Turnaway Effect:*** Longitudinal public health studies (such as the ***Turnaway Study***) have demonstrated that women who are denied access to legal abortion are significantly more likely to experience sustained economic hardship. They face a higher risk of falling below the poverty line, relying on state public assistance, and experiencing long-term unemployment compared to those who obtained the care they sought.
- ***Labor Market Participation:*** Legalization historically triggers an increase in women's workforce participation, particularly in high-skilled, professional occupations. By preventing unplanned, early-term disruptions, individuals are better positioned to complete higher education degrees and enter competitive career tracks, narrowing the gender wage gap over time.

Public Health, Maternal Mortality, and Safety

The legal status of abortion does not eliminate the demand for the procedure; rather, it dictates whether the procedure is performed safely under sanitary medical supervision or covertly under life-threatening conditions.

- ***Reduction in Maternal Mortality:*** In environments where abortion is heavily criminalized or banned, individuals frequently resort to unsafe, unregulated methods. This leads to a dramatic spike in preventable maternal mortality and severe morbidity, including systemic infections, hemorrhaging, and permanent infertility. Legalization allows public health systems to standardize clinical practices, driving down maternal death rates significantly.
- ***Mental Health and Well-being:*** Being denied a desired medical procedure under threat of criminal prosecution induces severe psychological distress. Conversely, legal access ensures that individuals receive compassionate, non-judgmental pre- and post-operative psychological support,

protecting long-term mental well-being and reducing the stigma that often isolates vulnerable patients.

Systemic Disparities and Marginalized Communities

While the restrictions of criminalization apply broadly on paper, their real-world consequences are disproportionately borne by specific demographic groups, creating a layered human rights crisis.

- ***The Wealth Gap in Access:*** Affluent individuals can often bypass restrictive domestic laws by traveling abroad or accessing high-cost, private, off-the-record medical networks. Low-income women, rural residents, and undocumented individuals, however, lack the financial resources, flexible work schedules, or child care necessary to travel, forcing them into dangerous situations.
- ***Minority and Youth Vulnerabilities:*** Racial minorities and young women often face compounding institutional barriers, including a lack of localized health clinics and targeted systemic discrimination within public healthcare sectors. Legalization serves as an essential baseline to dismantle these intersectional disparities.

f. Financial and Healthcare Accessibility

The legalization of abortion is only the first step in ensuring reproductive rights. A law that permits abortion on paper means very little if a woman cannot afford the procedure or cannot find a clinic that offers it. True accessibility requires that safe abortion services are both financially affordable and physically reachable for everyone, regardless of their income or where they live.

The Barrier of High Out-of-Pocket Costs

When abortion services are not covered by state healthcare or public insurance, the cost of the procedure becomes a major obstacle.

- ***Delayed Care:*** Low-income individuals often spend weeks trying to raise the necessary funds. This delay can push the pregnancy into a later term,

making the eventual medical procedure more complex, expensive, and difficult to access.

- ***The Cycle of Poverty:*** Forcing a woman to pay high out-of-pocket costs for essential healthcare can lead to sudden financial distress, forcing her to choose between medical care and basic needs like rent, food, or childcare.

Geographic Obstacles and Medical Deserts

Even if a country has legalized abortion, the physical distribution of healthcare providers heavily dictates who can actually receive care.

- ***Rural Isolation:*** Abortion clinics are overwhelmingly located in major urban centers. Women living in rural areas or small towns often face "medical deserts," meaning they must travel hours or even days to find a certified provider.
- ***Indirect Costs:*** Long-distance travel brings heavy indirect costs, including expensive transportation, hotel stays, lost wages from taking days off work, and the logistical challenge of finding temporary childcare.

Shortage of Trained Providers and Institutional Refusal

Physical accessibility is also limited by the number of medical professionals available and willing to perform the procedure.

- ***Stigma and Lack of Training:*** In many regions, medical schools do not include safe abortion methods in their standard training, leading to a permanent shortage of qualified doctors.
- ***Conscientious Objection:*** Many countries allow doctors or entire hospitals to refuse to perform abortions based on personal or religious beliefs. When widespread, this practice can leave entire geographic regions without a single active provider, forcing patients to look elsewhere.

g. Cases to Legalize Abortion

Even in jurisdictions with highly restrictive laws, international legal frameworks and human rights bodies generally identify a specific baseline of exceptional circumstances where abortion should be legalized. These critical cases represent situations where forcing an individual to carry a pregnancy to term directly endangers their life, health, or fundamental human dignity.

Danger to the Life or Health of the Pregnant Individual

This is the most widely recognized case for legal exception. It applies when continuing the pregnancy poses an immediate threat to the individual's survival or long-term physical and mental health. Conditions such as severe pre-eclampsia, cardiovascular failure, or cancer requiring immediate chemotherapy make the abortion a life-saving medical necessity.

Pregnancy Resulting from Sexual Violence (Rape and Incest)

Forcing a survivor of sexual assault, rape, or incest to carry a pregnancy to term is recognized by international human rights bodies as a form of cruel, inhuman, and degrading treatment. Legal exceptions in these cases focus on preserving the victim's psychological well-being, personal autonomy, and protecting them from severe, compounding trauma.

Severe or Fatal Fetal Abnormalities

This case involves pregnancies where medical imaging or genetic testing reveals severe, incurable fetal defects that mean the fetus cannot survive outside the womb or will face a severely compromised life. Legalizing abortion under these circumstances allows families to make compassionate medical choices and avoids prolonged emotional torture for the parents.

5. Critical Countries' Views on the Legal Status of Abortion and Reproductive Rights Under International Law

United States of America: The United States presents one of the most dynamic, heavily polarized, and legally complex landscapes regarding abortion, deeply rooted in its federalist structure and shifting domestic politics. For decades, US policy was anchored by the landmark 1973 Supreme Court decision *Roe v. Wade*, which recognized a constitutional right to abortion nationwide. However, the legal reality shifted dramatically with the 2022 *Dobbs v. Jackson Women's Health Organization* ruling, which overturned *Roe* and returned the authority to regulate or completely ban abortion to individual states.

This monumental shift dissolved national uniformity, dividing the country into highly volatile "*medical deserts*" and "*reproductive havens*." Across the nation, states have split into two extremes:

- **The Restrictive Bloc:** Roughly half of US states immediately enacted near-total bans or strict gestational limits (such as 6-week "heartbeat" bills), many with no exceptions for rape or incest. These states utilize severe criminal penalties, targeting healthcare providers with felony charges, prison time, and the revocation of medical licenses.
- **The Protective Bloc:** Conversely, states like California, New York, and Illinois have explicitly codified abortion access into their state laws and constitutions, actively positioning themselves as sanctuaries for out-of-state patients.

On the global stage, particularly within the United Nations, this domestic polarization directly dictates US foreign policy through a historical back-and-forth mechanism known as the "*Global Gag Rule*" (*the Mexico City Policy*). When a Republican administration is in power, the US restricts foreign aid from going to any international non-governmental organization (NGO) that performs or actively promotes abortion as a method of family planning. Conversely, Democratic administrations consistently revoke this policy to prioritize universal health-care integration. This political whiplash creates severe instability for UN-backed global health initiatives, particularly in developing nations that rely on USAID funding.

El Salvador: El Salvador stands as one of the most legally rigid nations in the world regarding fetal rights, enforcing a total ban on abortion under any circumstance. While narrow exceptions existed prior to 1998, a historic legislative overhaul eliminated all allowances, including cases of rape, incest, or direct

threats to maternal survival. This absolute doctrine is anchored by Article 1 of the Salvadoran Constitution, which explicitly recognizes the right to life **from the moment of conception**. Under this framework, individuals suspected of terminating a pregnancy are aggressively prosecuted, with charges frequently upgraded to aggravated homicide, carrying prison sentences of 30 to 50 years. This creates a severe chilling effect in public healthcare, forcing medical staff to report obstetric emergencies to law enforcement to avoid up to 12 years of imprisonment for suspected complicity.

On the international stage, El Salvador's unyielding stance makes it a frequent target of criticism within the United Nations and the Inter-American Human Rights system. UN bodies like **CEDAW** consistently urge the state to reform its penal code, while landmark litigation—such as **Beatriz v. El Salvador** before the Inter-American Court—directly challenges the absolute ban when a fetus is non-viable and the mother's life is endangered. In response, El Salvador consistently asserts its **territorial sovereignty**, defending its constitutional mandate against what it frames as foreign intervention.

Islamic Republic of Iran: The Islamic Republic of Iran approaches abortion through a precise framework shaped by Shi'ite Islamic jurisprudence (Fiqh) and state-driven demographic goals. Under the **Therapeutic Abortion Act**, abortion is permitted only within the first four months of pregnancy (before "Ensoulement") under strict medical criteria: when a specialized medical board certifies that the pregnancy directly threatens the mother's life or involves fatal fetal abnormalities. Beyond these narrow exceptions, abortion is strictly criminalized under the Islamic Penal Code, carrying severe penalties for providers and the mandate to pay **Diyah** (blood money) to the fetus's family.

In recent years, Iran's legal landscape has grown significantly more restrictive due to its shifting demographic priorities. To counter a rapidly aging population, the state enacted the **"Youthful Population and Protection of the Family" Law**, which aims to maximize birth rates by restricting reproductive healthcare. This law severely limits access to contraceptives, bans public healthcare networks from offering free family planning, and heavily penalizes the unauthorized distribution of abortion medications.

On the international stage, particularly within the United Nations, Iran maintains a staunchly conservative alignment. The Iranian delegation consistently rejects Western-led human rights doctrines that interpret "reproductive rights" as universal mandates. Citing **cultural relativism**, Iran argues that UN resolutions

must respect the diverse religious foundations, traditional family structures, and sovereign domestic laws of individual member states.

Kingdom of the Netherlands: The Kingdom of the Netherlands stands as one of the world's most progressive and vocal champions of reproductive freedom, personal autonomy, and gender equality. Domestically, the Netherlands has a highly liberal and comprehensive legal framework governed by the Termination of Pregnancy Act. Under Dutch law, abortion is completely legal on request up to the point of fetal viability (traditionally set at 24 weeks). To ensure true equity and eliminate financial obstacles, the procedure is fully integrated into the state's universal healthcare system, meaning it is provided entirely free of charge to Dutch residents through national insurance schemes. Furthermore, to combat geographic barriers and modern restrictions, the Netherlands legally permits the use of telemedicine for early-term medical abortions and has systematically removed bureaucratic hurdles, such as the historic mandatory five-day waiting period, to ensure immediate, dignified care.

On the international stage, particularly within the United Nations General Assembly and the Sixth Committee, the Netherlands is a powerful, proactive leader in codifying reproductive rights as fundamental human rights. The Dutch delegation consistently rejects arguments based on cultural relativism that seek to dilute women's rights, asserting instead that access to safe, legal healthcare is an unshakeable, universal mandate. Furthermore, the Netherlands serves as a global financial and diplomatic counterweight to restrictive global policies. When foreign aid is restricted by other global powers through mechanisms like the "Global Gag Rule," the Netherlands routinely leads massive multilateral funding initiatives, such as the global **"She Decides" movement**, to ensure that UN agencies, global healthcare networks, and cross-border NGOs can continue to distribute contraceptives and provide safe abortion access in developing nations.

United Kingdom: The United Kingdom operates under a highly progressive, robustly regulated, and universally accessible framework for reproductive rights, positioning it as a key global defender of bodily autonomy and gender equality. In Great Britain, abortion is governed by the **Abortion Act 1967**. While the procedure technically remains under criminal law unless specific statutory exemptions are met, in practice, abortion is fully legal up to 24 weeks of pregnancy if two registered medical practitioners certify that continuing the pregnancy in-

volves a risk to the physical or mental health of the woman. For fetal abnormalities or direct threats to the mother's life, there is no gestational time limit.

To ensure true accessibility, abortion is entirely integrated into the state-funded **National Health Service (NHS)**, removing all out-of-pocket financial barriers for residents. Furthermore, the UK has been a pioneer in deploying modern public health strategies: during the COVID-19 pandemic, the government permanently legalized **"pills by post" (telemedicine)**, allowing individuals to receive early-term medical abortion medications at home following a remote consultation, a move that drastically reduced geographic barriers and waiting times.

On the international stage, particularly within the United Nations and the Sixth Committee, the UK is a steadfast and vocal champion of Sexual and Reproductive Health and Rights (SRHR). The British delegation consistently treats reproductive freedom not as a matter of domestic policy preference, but as a core component of **universal human rights, gender equity, and global sustainable development**. The UK works actively through the UN to counter restrictive trends and regularly uses its international development funding to support global health agencies, championing cross-border medical networks and the integration of safe abortion care into humanitarian aid responses.

China: The People's Republic of China approaches the issue of reproductive rights and abortion through a distinct lens centered on **state-guided demographic policy, socioeconomic planning, and secular public health management**. Unlike nations where the abortion debate is deeply intertwined with religious jurisprudence or personal moral philosophies, China treats reproductive legislation primarily as an instrument of state planning and macro-population control. Historically, during the era of the One-Child Policy (1979–2015), abortion was widely accessible and heavily utilized by the state to meet strict population caps. In the post-2015 era, faced with a rapidly aging society and plummeting birth rates, China transitioned to a "Three-Child Policy." Consequently, the state's legal and medical priorities have shifted toward **optimizing population quality and reducing "non-medically necessary" abortions** to encourage family growth.

Despite these shifting demographic targets, abortion remains entirely legal and widely accessible under China's national healthcare framework. Governed by the **Law on the Protection of Women's Rights and Interests** and the **Maternal and**

Infant Health Care Law, terminations are fully integrated into public health systems. China has long been a pioneer in medical abortions, heavily utilizing mifepristone and misoprostol regimens since the late 1980s, which drastically reduced maternal mortality rates from unsafe procedures. However, under recent State Council directives—such as the ***Guidelines on the Development of Chinese Women (2021–2030)***—the government has implemented strict medical oversight to curb high rates of repeat abortions among young people. This includes enforcing comprehensive post-abortion family planning services, mandating contraceptive education, and restricting sex-selective abortions (a measure heavily enforced to combat historical gender imbalances).

On the international stage, particularly within the United Nations General Assembly and the Sixth Committee, China is a staunch defender of ***absolute state sovereignty and non-interference in domestic affairs***. The Chinese delegation consistently maintains that population policies, healthcare frameworks, and family planning laws are strictly within the domestic jurisdiction of each sovereign nation. China rejects any attempt by external bodies or multilateral resolutions to impose universal moral mandates or Western-centric human rights frameworks on sovereign states. Within the UN, China balances its stance by supporting global public health initiatives that reduce maternal mortality, while firmly opposing any international legal mechanisms that threaten a state's absolute right to govern its own territory and manage its demographic structures.

6. Topics a Resolution Should Address

- 1.** What kind of supplementary budget is required to guarantee secure and healthy access to abortion and reproductive rights?
- 2.** Is it correct for the State to hold regulatory and oversight authority over abortion rights, or should this decision be left entirely to personal choice and medical professionals?
- 3.** If a citizen gets a legal abortion abroad and returns to a country where it is banned, can their home country put them on trial? How should international law handle this conflict?
- 4.** How can the UN set basic global rules for reproductive rights without violating the independence and laws of member states?
- 5.** Is it a violation of national sovereignty when foreign NGOs fund hidden abortion networks inside a country, or is it just humanitarian aid?
- 6.** Under what specific exceptions such as rape, incest, or medical emergencies must abortion be guaranteed as a universal legal right?
- 7.** How can international law create a unified legal framework for reproductive rights when member states are fundamentally divided on the definition of fetal personhood?
- 8.** What public health and legal mechanisms should member states provide to mitigate the medical and psychological impacts of abortion on women?

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